

MEMORANDUM

TO: Pharmacy and Therapeutics Committee
FROM: Hari Acharya, Healthcare Analytics
DATE: May 25, 2026
RE: Recommended utilization management interventions on high spend branded drugs with generic alternatives

Summary

The Medicare Part D dataset surfaces three specific utilization management opportunities totaling approximately 4.2 billion dollars in potential annual savings. Each targets a high spend brand name molecule with an existing generic alternative and a current Generic Dispensing Rate well below the 90 percent industry benchmark. Insulin opportunities, although larger in absolute spend, are excluded from this round because rebate contracts limit achievable savings without renegotiation. A phased rollout is proposed below to manage member disruption.

Proposed Interventions

Intervention	Current GDR	Target GDR	Annual Savings
1. Lenalidomide step therapy	36.0%	90.0%	\$2.9 billion
2. Tiotropium step therapy	1.7%	60.0%	\$0.8 billion
3. Aripiprazole PA tightening	93.5%	98.0%	\$0.5 billion
Total			~\$4.2 billion

1. Lenalidomide step therapy

Brand Revlimid spending is approximately 3.86 billion dollars per year at a 36 percent generic dispensing rate. Lenalidomide generics received FDA AB rated bioequivalence approval in late 2022, making this the freshest and largest LOE opportunity in Part D. Recommendation: implement step therapy requiring a documented trial of generic lenalidomide before brand Revlimid is approved. Maintain an oncologist override pathway for documented intolerance or clinical concern, with priority handling given the multiple myeloma indication.

2. Tiotropium step therapy

Brand Spiriva spending is approximately 1.71 billion dollars at a 1.7 percent generic dispensing rate. Generic tiotropium received approval in 2024 for the HandiHaler delivery format only. Recommendation: implement step therapy requiring generic tiotropium before brand Spiriva HandiHaler. Spiriva Respimat, the soft mist inhaler format, has no generic equivalent and should be handled separately. Members already stabilized on either device should be grandfathered to avoid disruption in the COPD population.

3. Aripiprazole prior authorization tightening

Brand spending is approximately 671 million dollars at a 93.5 percent overall GDR. Most remaining brand spend concentrates in Abilify Maintena, the long acting injectable formulation. Recommendation: tighten prior authorization criteria for brand oral aripiprazole to require documentation of generic intolerance or therapeutic failure. Maintain a separate PA pathway for Abilify Maintena, which is clinically distinct from oral generic aripiprazole and should not be subject to the same step therapy.

Excluded from this round

Insulin Aspart, Lispro, and Degludec combined represent over 6 billion dollars in branded spend with effectively zero generic dispensing. The cause is rebate locked contracting between manufacturers and the plan, not a clinical limitation. UM intervention without simultaneous contract renegotiation produces minimal savings. Refer to the contracting team for a separate workstream.

Phased rollout

Quarter one. Pilot Lenalidomide step therapy on commercial members. Collect adherence and exception data to refine override criteria. Quarter two. Extend Lenalidomide to Medicare members with full appeals documentation. Begin Tiotropium step therapy on commercial members. Quarter three. Extend Tiotropium to Medicare. Begin Aripiprazole PA tightening with grandfathering for stable members. Quarter four. Review Q1 through Q3 data and adjust criteria based on adherence, member disruption metrics, and actual savings realized.

Member disruption mitigation

All interventions include a physician override pathway with documented clinical justification. Member appeals process timing meets CMS requirements of 24 hours for urgent requests and 72 hours for standard. A pharmacist override is available at the point of sale for established therapy, allowing dispensing while documentation is gathered.

Analysis built on 71,545 rows of CMS Medicare Part D Spending by Drug data covering 2019 through 2023. Savings estimates use a 10 to 1 brand to generic price ratio. Live simulator and full methodology at partd-analytics-hari.streamlit.app.